

SOUTH EAST REGIONAL HEALTH AUTHORITY DATA QUALITY AUDIT REPORT 2026

Strategic Information Component
MINISTRY OF HEALTH AND WELLNESS
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Executive Summary

The 2026 HIV Prevention Data Quality Audit (DQA) for the South East Regional Health Authority (SERHA) assessed the quality, completeness, and reliability of prevention data reported in the District Health Information System 2 (DHIS2) against source outreach registers. The audit reviewed eleven (11) core indicators related to client demographics, outreach activities, HIV and syphilis testing, and HIV self-test distribution. A total of 5,217 entries from January–September 2025 were selected for audit, of which 4,387 (84%) were presented for verification, representing an improvement from 80% in 2025.

The audit found substantial improvements in the accuracy of several key indicators. Matching rates for HIV test results increased from 89% to 99%, syphilis results from 87% to 93%, and most demographic variables exceeded 98% accuracy. The newly assessed first-name indicator achieved 97%, surpassing the national target of 95%. These findings demonstrate strengthened data capture practices and greater consistency in recording critical client-level information, which enhances the reliability of programme monitoring and reporting.

Despite these gains, the audit identified significant weaknesses that affected overall performance. The proportion of outreach register entries fully aligned with DHIS2 declined from 87% in 2025 to 67% in 2026, while overall data accuracy decreased from 87% to 82%. These findings point to persistent challenges in data entry, reconciliation, and the timely upload of outreach records into DHIS2. Across all parishes, audits identified outreach and HIV self-test records that were documented in source registers but had not been entered into the electronic reporting system, resulting in discrepancies between reported and actual service delivery data.

The implications of these findings are significant. Incomplete or unmatched records compromise the integrity of programme data, potentially leading to underreporting of services delivered, inaccurate assessment of programme performance, and suboptimal resource allocation. Reduced confidence in reported data may also limit the programme’s ability to accurately monitor coverage, identify service gaps, and demonstrate progress toward national HIV prevention targets. Furthermore, discrepancies between source documents and DHIS2 increase the risk of inaccurate strategic and operational decision-making.

While data management and storage practices were generally strong, achieving a regional score of 94%, operational challenges remained evident. These included staffing gaps, particularly the absence of a Behaviour Change Communication Officer in St. Catherine, limitations in data entry capacity, and weaknesses in routine data quality verification processes. These factors contributed to delays in data entry, incomplete uploads, and reduced audit performance.

Overall, SERHA received a DQA compliance score of 67%, resulting in a “Needs Improvement” rating. Parish-level scores ranged from 66% in St. Thomas to 70% in St. Catherine, indicating that data quality challenges are region-wide rather than isolated (See table 1). Although improvements in indicator-level accuracy demonstrate progress in data collection practices, the decline in data

completeness, register-to-DHIS2 alignment, and overall audit scores highlights the need for strengthened data reconciliation processes, routine quality assurance activities, enhanced staff capacity, and timely entry of all outreach records into DHIS2. Addressing these issues will be critical to ensuring that prevention programme data accurately reflect service delivery and can effectively support evidence-based planning, monitoring, and decision-making.

Table 1: Data Quality Compliance Rating 2026

Parish	DQA Score	Compliance Rating
St Thomas	66%	Needs Improvement
KSA	68%	Needs Improvement
St Catherine	70%	Needs Improvement
SERHA	67%	Needs Improvement